

PSYCHODIAGNOSTIC ASSESSMENT REPORT

Brief Psychiatric Rating Scale (BPRS) & Sacks Sentence Completion Test (SSCT)

IDENTIFYING INFORMATION

Name: _____
Age / Sex: _____ / _____
Education: _____ Occupation: _____
Marital Status: _____ Date of Testing: _____
Informant / Referral: _____
Examiner: _____

TESTS ADMINISTERED & RATIONALE

Brief Psychiatric Rating Scale (BPRS)

The BPRS was administered to obtain a clinician-rated evaluation of the overall severity and range of psychiatric symptomatology. It surveys domains such as anxiety, depression, hostility, suspiciousness, unusual thought content, hallucinatory behaviour, and psychomotor disturbance, providing a broad cross-sectional index of symptom burden and a baseline against which treatment response can be monitored.

Sacks Sentence Completion Test (SSCT; Sacks & Levy, 1950)

The SSCT was administered to explore the client's conscious and pre-conscious attitudes, conflicts, and emotional concerns across four broad areas - Family, Sex, Interpersonal Relationships, and Self-Concept - encompassing fifteen attitude categories. As a semi-structured projective method, it yields clinically meaningful leads regarding areas of emotional adjustment and maladjustment that may not be directly verbalised during interview, and helps identify priority areas for therapeutic intervention.

BEHAVIOURAL OBSERVATIONS

The client was [cooperative / apprehensive] during administration, and rapport was established [easily / with some difficulty]. Instructions were understood adequately. The test was completed in approximately _____ minutes. [Note any hesitations, omissions, erasures, latency on specific items, and emotional reactions such as sighing, laughter, or discomfort.] The client appeared [relaxed / anxious / guarded] through the course of the assessment.

RESULTS AND INTERPRETATION

A. Brief Psychiatric Rating Scale (BPRS)

Obtained Score	Interpretation
40	Moderate level of overall psychiatric symptomatology

A total score of 40 on the BPRS falls in the moderate range of overall psychopathology, indicating a clinically meaningful level of symptom burden that is above minimal/very-mild presentations but below the marked-to-severe range typically associated with florid or grossly disorganised states. The score is consistent with the presence of active symptoms across one or more domains (e.g., affective, anxious, or thought-related) that are interfering with functioning and warrant clinical attention and intervention. As the BPRS is a global severity index, the specific symptom pattern should be read from the individual item ratings and integrated with history, mental status examination, and the projective findings below. The score also establishes a quantitative baseline for monitoring change over the course of treatment.

B. Sacks Sentence Completion Test (SSCT) - Quantitative Summary

Each of the fifteen attitude areas is rated for degree of disturbance (0 = no significant disturbance; 1 = mild disturbance, conflicts present but manageable without therapeutic aid; 2 = severe disturbance, appears to require therapeutic aid). Area ratings below are derived from the pattern across the four items constituting each area. The raw item-level sum across all sixty items is reported alongside for completeness.

Domain	Attitude Area	Items	Item Sum	Area Rating (0-2)
I. FAMILY	Attitude toward Mother	14,29,44,59	3	1 - Mild disturbance
	Attitude toward Father	1,16,31,46	6	2 - Severe disturbance
	Attitude toward Family Unit	12,27,42,57	1	0 - No significant disturbance
II. SEX	Attitude toward Women/Men	10,25,40,55	3	1 - Mild disturbance
	Attitude toward Heterosexual Relationships	11,26,41,56	1	0 - No significant disturbance
III. INTERPERSONAL	Attitude toward Friends & Acquaintances	8,23,38,53	2	1 - Mild disturbance
	Attitude toward Superiors	6,21,36,51	3	1 - Mild disturbance
	Attitude toward People Supervised	4,19,34,49	1	0 - No significant disturbance
	Attitude toward Colleagues	13,28,43,58	0	0 - No significant disturbance
IV. SELF-CONCEPT	Fears	7,22,37,52	4	1 - Mild disturbance
	Guilt Feelings	15,30,45,60	3	1 - Mild disturbance
	Goals	3,18,33,48	5	2 - Severe disturbance
	Attitude toward Own Abilities	2,17,32,47	5	2 - Severe disturbance
	Attitude toward Past	9,24,39,54	1	0 - No significant disturbance
	Attitude toward Future	5,20,35,50	3	1 - Mild disturbance

Area-rating total: 13 / 30 (standard Sacks 15-area disturbance total).

Raw item-level sum: 41 / 120 (sum of all sixty 0-2 item ratings). Note: the itemised ratings supplied sum to 41; a stated total of 46 was also noted by the referrer. As the individual item values are the primary data, the computed value of 41 is used here, and the 5-point discrepancy is flagged for verification.

Domain-level picture

Domain	Item Sum	Relative Loading
I. Family	10	Elevated - driven mainly by the paternal relationship
II. Sex	4	Low - largely unremarkable
III. Interpersonal	6	Mild - some strain with authority figures
IV. Self-Concept	21	Most disturbed domain overall

C. Qualitative Analysis (Area-wise)

Attitude toward Father (Items 1,16,31,46) - Rating: 2 (Severe disturbance)

This area emerges as SEVERELY disturbed and is the single most conflicted relationship in the protocol. The responses converge on themes of dissatisfaction, unmet expectation, and a wish for the father to be different, suggesting a strained, ambivalent, or emotionally distant paternal bond. Given the weight of this area, it is likely to be a central organising conflict and a priority focus for intervention and family-oriented work.

Goals (Items 3,18,33,48) - Rating: 2 (Severe disturbance)

The goal-related items indicate SEVERE disturbance, pointing to conflict around ambitions, unfulfilled wishes, and a sense that satisfaction is contingent on conditions that feel out of reach. This suggests difficulty in translating aspiration into attainable, self-directed objectives.

Attitude toward Own Abilities (Items 2,17,32,47) - Rating: 2 (Severe disturbance)

Responses reflect SEVERE disturbance in self-efficacy, with prominent self-doubt, awareness of perceived weakness, and uncertainty about coping when circumstances turn unfavourable. Together with the Goals finding, this indicates a fragile, self-critical self-concept - a core self-perception vulnerability.

Attitude toward Mother (Items 14,29,44,59) - Rating: 1 (Mild disturbance)

Mild disturbance is noted. The maternal relationship appears comparatively less conflicted than the paternal one, though some ambivalence or unmet need is present.

Attitude toward Women/Men (Items 10,25,40,55) - Rating: 1 (Mild disturbance)

Mild disturbance, with some critical or guarded appraisal of the opposite sex; not a pervasive area of conflict.

Attitude toward Friends & Acquaintances (Items 8,23,38,53) - Rating: 1 (Mild disturbance)

Mild disturbance, suggesting some selectivity, dissatisfaction, or wariness in peer relationships without marked social withdrawal on this measure.

Attitude toward Superiors (Items 6,21,36,51) - Rating: 1 (Mild disturbance)

Mild disturbance toward authority figures. Read together with the severe paternal conflict, this raises the possibility of a broader authority-related theme worth exploring clinically.

Fears (Items 7,22,37,52) - Rating: 1 (Mild disturbance)

Mild-to-moderate disturbance, with identifiable fears that the client is somewhat reluctant to disclose and that may at times influence behaviour.

Guilt Feelings (Items 15,30,45,60) - Rating: 1 (Mild disturbance)

Mild disturbance, indicating some guilt and self-reproach linked to past actions or perceived mistakes.

Attitude toward Future (Items 5,20,35,50) - Rating: 1 (Mild disturbance)

Mild disturbance, reflecting a mixed or uncertain outlook toward the future - neither strongly optimistic nor uniformly pessimistic.

Areas rated 0 (no significant disturbance): Family Unit, Heterosexual Relationships, People Supervised, Colleagues, and Past. These represent relatively conflict-free zones and potential sources of strength.

D. Interrelationship of Attitudes

The disturbance is concentrated in two convergent clusters. First, a self-concept cluster (Goals and Own Abilities both severe, with Future mildly affected) points to a self-critical, low-efficacy self-view and difficulty setting attainable goals. Second, an authority/paternal cluster (Father severe; Superiors mild) suggests that conflict originating in the paternal relationship may generalise to other authority figures. The comparative sparing of the Sex domain, Family Unit, and Past indicates that the difficulties are focal rather than pervasive, which is prognostically favourable.

SUMMARY AND INTEGRATION

Taken together, the BPRS indicates a moderate overall level of psychiatric symptomatology requiring active clinical attention, while the SSCT localises the emotional difficulty to specific, clinically coherent areas rather than a diffuse global disturbance. The projective data most strongly implicate the paternal relationship and the self-concept domain (goals and perceived abilities), with a secondary authority theme. The client retains several conflict-free areas (family unit, past, colleagues, heterosexual relationships), which represent ego resources and can be leveraged in therapy. The BPRS severity and the focal SSCT conflicts are mutually consistent and should be integrated with the clinical history and mental status examination for a complete formulation.

RECOMMENDATIONS

- Integrate these findings with the clinical interview, history, and mental status examination before arriving at a formulation and diagnosis.
- Prioritise the paternal-relationship and self-concept conflicts (goals, own abilities) in psychotherapy; consider family-oriented sessions where indicated.
- Address the self-critical, low-efficacy self-view through cognitive and behavioural strategies (cognitive restructuring, graded goal-setting, and mastery experiences).
- Explore the authority theme (father / superiors) as a possible generalising pattern.
- Re-administer the BPRS periodically to monitor symptom change against the current baseline of 40.
- Verify the SSCT total: itemised ratings sum to 41/120 whereas 46 was also reported - reconcile before finalising the record.

Assessed & Reported by: _____ Supervised by: _____

Date: _____ Signature: _____